

UC Flare

Survival Guide

What to do when ulcerative colitis symptoms spike

What's inside:

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This guide is for educational purposes only. It is not medical advice, diagnosis, or treatment. Always consult a licensed gastroenterologist for your specific situation.

Understanding a UC Flare

A flare — also called active disease or a relapse — occurs when UC symptoms return or worsen after a period of remission. During remission, inflammation in the colon is low or absent and most people function relatively normally. During a flare, that inflammation returns, bringing symptoms with it.

Flares can be mild, moderate, or severe. Some resolve within a few weeks with the right treatment. Others last months if left unaddressed. Recognizing one early and contacting your gastroenterologist promptly gives you the best chance of getting it under control quickly.

Contact your GI doctor when you notice any of the following returning or worsening:

- More trips to the bathroom than your usual baseline
- Return of blood in stool after it had been absent
- More abdominal cramping or pain than normal
- Looser or more urgent stools
- Fatigue that worsens despite adequate rest
- Waking at night to use the bathroom
- Loss of appetite or nausea

Tip: Track your symptoms daily — stool frequency, blood (yes/no), pain level 1–10, and energy level. This information is exactly what your doctor needs to assess your flare severity.

Seek emergency care immediately if you have heavy rectal bleeding, severe abdominal pain, high fever (above 101.3°F / 38.5°C), signs of dehydration, or inability to keep fluids down.

Call your doctor same-day (do not wait for your next appointment) if you have:

- Significantly increased rectal bleeding
- Fever above 101.3°F / 38.5°C
- Severe abdominal pain or cramping
- Feeling faint, very weak, or dizzy
- Inability to eat or drink for more than 24 hours
- Rapid weight loss over a short period

Missing or stopping medication

Missing doses of UC medication — even briefly — is one of the most common causes of a flare. Mesalamine and other maintenance medications must be taken consistently, not just when you have symptoms.

Infections and illness

Gastrointestinal infections (including *C. difficile*) can trigger flares directly. If you develop a stomach bug during your flare, ask your doctor to test for infection before changing your UC medications.

NSAIDs (ibuprofen, naproxen)

Ibuprofen (Advil, Motrin) and naproxen (Aleve) can irritate the gut lining and worsen UC. Use acetaminophen (Tylenol) instead for pain — and always check with your doctor first.

Antibiotics

Antibiotics disrupt the gut microbiome and can trigger flares in some people. Always inform your gastroenterologist if you are prescribed antibiotics for any reason.

Psychological stress

Stress does not cause UC but can worsen symptoms. Sleep, regular movement, and mental health support are all part of managing UC effectively.

Certain foods

No food causes UC, but during a flare specific foods aggravate inflammation. See the diet section of this guide for specifics.

What to Eat During a Flare

When UC symptoms spike, the inflamed colon is more sensitive and has less capacity to process difficult foods. Eating thoughtfully reduces gut irritation and helps maintain nutrition while your body heals. The goal is not perfection — it is minimizing stress on your digestive system while staying nourished.

Eat small, frequent meals rather than three large ones. Small portions put less strain on the inflamed gut and reduce cramping after eating.

Cooked vegetables (peeled and soft)

- Peeled, well-cooked carrots
- Peeled mashed or boiled potatoes (no skin)
- Cooked zucchini or squash (no seeds)
- Butternut squash, steamed until soft
- Cooked green beans (no skins)

Refined grains and starches

- White rice — plain, well cooked
- Plain white bread or toast
- Plain crackers (saltines)
- Plain pasta or white-flour noodles
- Plain oatmeal — cooked soft

Lean proteins

- Skinless chicken breast — baked or boiled
- White fish (cod, tilapia) — baked or steamed
- Eggs — scrambled, boiled, or poached (not fried)
- Firm tofu (plain)
- Smooth peanut butter in small amounts (if tolerated)

Soft fruits (ripe and peeled)

- Ripe banana — one of the most universally tolerated fruits
- Canned peaches or pears in juice (no skin)
- Unsweetened cooked applesauce
- Ripe melon (no rind)

Hydrating liquids

- Water — sip consistently throughout the day
- Oral rehydration solutions (Pedialyte, Liquid IV) — replace electrolytes lost through diarrhea
- Clear chicken or vegetable broth
- Bone broth
- Herbal teas (chamomile, peppermint — not caffeinated)
- Diluted fruit juice (not citrus)

' Raw vegetables with skins

High insoluble fiber — hard to digest, increases stool frequency

- ' **Whole grains, bran, seeds, nuts**
High fiber increases stool bulk and urgency
- ' **Spicy foods**
Irritates the gut lining and worsens urgency and cramping
- ' **Fried or fatty foods**
Stimulates gut motility and makes diarrhea worse
- ' **Caffeine (coffee, energy drinks)**
Stimulates bowel movements, increases urgency
- ' **Alcohol**
Irritates the gut lining and can worsen inflammation
- ' **Artificial sweeteners**
Sugar alcohols (sorbitol, mannitol) cause diarrhea and gas
- ' **Carbonated drinks**
Cause bloating and gas in an already inflamed gut
- ' **Large portions**
Overwhelm digestive capacity — eat small amounts more frequently
- ' **Dairy (if sensitive)**
Can worsen loose stools in people with lactose sensitivity — test your tolerance

Managing a Flare at Home

- Stay hydrated — diarrhea causes fluid and electrolyte loss. Oral rehydration solutions are better than water alone during severe diarrhea.

- Eat small, low-irritation meals as described in the diet section. Do not skip meals entirely even if appetite is low.
- Rest when possible — fatigue during a flare is real and has a physiological cause (inflammation, blood loss, disrupted sleep).
- Track your symptoms daily — stool frequency, blood present (yes/no), pain level 1–10, energy level. Your doctor will ask for this.
- Continue your maintenance medications unless your doctor tells you otherwise. Stopping them can make the flare worse.
- Use acetaminophen (Tylenol) if you need pain relief — avoid ibuprofen and naproxen, which can worsen UC.
- Contact your GI office and explain what is happening. Do not simply wait for your next scheduled appointment.

Do NOT stop your UC medications

Stopping mesalamine, biologics, or immunomodulators without guidance can cause the flare to worsen significantly.

Do NOT take ibuprofen or naproxen

NSAIDs irritate the gut lining and can trigger or worsen a flare. Use acetaminophen instead.

Do NOT wait weeks to contact your doctor

Flares are easier to control when caught early. Call your GI office as soon as you notice symptoms worsening.

Do NOT make major diet changes without guidance

Eliminating entire food groups can cause nutritional deficiencies on top of an already difficult flare.

Do NOT self-prescribe steroids

Prednisone and other steroids require medical supervision. Taking them incorrectly can cause serious side effects.

Do NOT ignore warning signs

Heavy bleeding, high fever, severe pain, or inability to keep fluids down are emergencies. Seek care immediately.

Your gastroenterologist will assess flare severity and recommend treatment based on your individual history. Common approaches:

Mild to moderate flares

Increased dose of current medication, adding rectal therapy (suppositories or enemas), or a short course of oral corticosteroids (prednisone or budesonide).

Moderate to severe flares

Oral or intravenous corticosteroids, stepping up to a biologic medication (infliximab, vedolizumab, ustekinumab, or others) or a JAK inhibitor (tofacitinib, upadacitinib).

Refractory flares

If multiple medications have failed, your doctor may discuss surgical options. For UC, surgery (removal of the colon) can be curative — unlike Crohn's disease.

If you are repeatedly needing steroids to control flares, this is called steroid-dependence. It is a signal that a more effective long-term maintenance therapy is needed. Discuss this pattern with your GI doctor.

25 Questions to Ask Your GI Doctor

Bring this list to your next appointment. Being specific about your symptoms helps your doctor make the right decisions faster.

About your current flare

1. Do my current symptoms indicate I am in a flare — and how severe is it?
2. Do we need tests to confirm: stool test, blood test, or a scope?
3. How do my symptoms compare to my previous flares?
4. Is there any chance this could be a C. difficile infection rather than a UC flare?
5. Should I be checked for anemia given the blood in my stool?

About treatment

1. Do we need to adjust my current medication — dose, formulation, or route?
2. Is a short course of corticosteroids (prednisone or budesonide) appropriate?
3. If I need steroids again, does that mean my current treatment is not working?
4. Am I a candidate for a biologic medication or a JAK inhibitor?
5. What does each treatment option involve — how is it given, how often, what monitoring is required?
6. Are there patient assistance programs for biologics if cost is a concern?

About symptoms and monitoring

1. What specific symptoms should prompt me to call your office urgently?
2. What symptoms mean I should go to the emergency room?
3. Should I keep a symptom diary — and what should I track?
4. Should I have a colonoscopy to assess my current disease activity?
5. Is my fatigue related to anemia, and should we check my iron levels?

About diet and lifestyle

1. Should I be referred to a registered dietitian who specializes in IBD?
2. Are there specific foods I should avoid given my current medications?
3. Are there supplements I should be taking — iron, vitamin D, B12, folate?
4. Is it safe for me to exercise during this flare?

About long-term management

1. If this is my second (or third) flare this year, what does that mean for my long-term treatment plan?
2. Is my disease progressing — and how can we tell?
3. What are my options if I continue to flare on my current medication?
4. Should I consider clinical trials for new UC treatments?

Daily Symptom Tracker

Fill this in every day during your flare. Bring it to your appointment — your doctor will use this to assess severity and guide treatment decisions.

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Ulcerative Colitis Symptoms Guide

colitishelpusa.com/blog/ulcerative-colitis-symptoms

UC Treatment Options — Plain English

colitishelpusa.com/blog/ulcerative-colitis-treatment-options

Biologics for UC — Full Guide

colitishelpusa.com/blog/biologics-for-ulcerative-colitis

JAK Inhibitors for UC

colitishelpusa.com/blog/jak-inhibitors-ulcerative-colitis

Crohn's Disease vs Ulcerative Colitis

colitishelpusa.com/blog/crohns-vs-ulcerative-colitis

UC Diet Guide

colitishelpusa.com/blog/ulcerative-colitis-diet

UC Care Options Check (Free, 8 questions)

colitishelpusa.com/uc-care-options-check

Always consult a licensed gastroenterologist for decisions about your care.